Recent Tier 2 papers — not yet indexed (needs review)

- **Cycle:** 2026-07-03 | **Window:** 2026-06-19 to 2026-07-03
- **Tier 2 journals scanned; items found:** 3

> These are recent papers from the approved **Tier 2** journals that the strict, MeSH-verified digest does not yet catch because PubMed has not finished indexing them (no MeSH terms or publication types assigned yet). They are matched by journal + publication date + disease name (and, unless the broad option was used, phase II/III-trial or guideline wording in the title/abstract). Case reports/series and preclinical/animal studies are filtered out. They are **not** AI-appraised, and the study type is inferred from wording, not verified. Treat as a heads-up list to scan manually; verify each against the source.

Diffuse Large B-Cell Lymphoma

Pola-R-CHP as frontline therapy for diffuse large B-cell lymphoma: a systematic review and meta-analysis of randomized trials and real-world evidence.

Journal: Blood cancer journal | **Date:** 2026-Jun-26 | **PMID:** 42362499 | **DOI:** 10.1038/s41408-026-01552-5

Publication types: Journal Article

Abstract:

Pola-R-CHP regimen is the new standard of care for untreated diffuse large B-cell lymphoma (DLBCL) patients, following the POLARIX trial. This study evaluates whether clinical trial (RCT) efficacy and safety are reproducible in real-world evidence (RWE) settings. This systematic review and meta-analysis compared outcomes between RCTs and RWE studies. Primary endpoints were complete response (CR) rate, progression-free survival (PFS), and overall survival (OS). Meta-regression explored heterogeneity using age, International Prognostic Index (IPI), and activated B-cell-like (ABC) subtype as moderators. Data from 2 RCTs and 10 RWE studies (3472 patients) were integrated. The pooled CR rate was 78% (95% CI: 74-82%), with RWE studies showing a significantly higher CR (80%) than RCTs (69%, P < 0.001). The pooled 1-year PFS was 85%, significantly higher in RWE (86%) than RCTs (81%, P = 0.02). Pooled 1-year OS was 91%, with no significant difference between RWE (90%) and RCTs (94%, P = 0.35). Despite high heterogeneity (I

Short ramp-up glofitamab halves mortality risk after anti-CD19 CAR T-cell therapy failure in patients with diffuse large B-cell lymphoma: final results of the LYSA BiCAR phase 2 trial with a pre-specified external control arm.

Journal: Journal of hematology & oncology | **Date:** 2026-Jul-01 | **PMID:** 42393723 | **DOI:** 10.1186/s13045-026-01816-4

Publication types: Journal Article

Abstract:

BACKGROUND: Failure after anti-CD19 chimeric antigen receptor (CAR) T-cell therapy in diffuse large B-cell lymphoma (DLBCL) is associated with poor survival and there is no established standard of care. We previously reported the phase 2 LYSA BiCAR trial of short-ramp-up glofitamab after CAR T-cell failure. Here, we present the final survival results and a pre-specified external comparative effectiveness analysis against a contemporary control arm constructed from academic data.
METHODS: BiCAR is a multicenter, single-arm trial in adults with CD20-positive DLBCL refractory to, or in first relapse/progression after anti-CD19 CAR T-cell therapy. Participants received obinutuzumab pretreatment followed by intravenous glofitamab with an accelerated step-up to 30 mg within 8 days, then 30 mg every 21 days for up to 11 cycles. For comparative analyses, we constructed an external control arm from patients included in the French DESCAR-T registry and the ALYCANTE phase 2 trial who experienced CAR T-cell failure, who subsequently started

non-bispecific systemic therapy, met key BiCAR eligibility criteria, and initiated treatment within a ±1-month window around the BiCAR treatment period. To minimise datasource bias, both arms, glofitamab (BICAR) and control, were constructed from individual patient data from the DESCAR-T registry and the ALYCANTE trial. A propensity score including major prognostic variables was estimated and applied using stabilized inverse probability of treatment weighting with multiple imputation. Additional weighting schemes and restrictions were applied in sensitivity analyses, and restricted mean survival time (RMST) was evaluated by treatment arm.
RESULTS: Among 47 enrolled BiCAR patients, 46 received glofitamab. At a median follow-up of 30.4 months, median overall survival (OS) was 17.3 months, and the 2-year OS rate was 38.3%. For comparative effectiveness, 45 glofitamab-treated patients and 133 controls formed the analysis cohort. In the primary weighted analysis, median OS was 19.6 months for glofitamab and 7.6 months for controls, with a hazard ratio for death of 0.49 (95% CI, 0.31-0.79; P = 0.007). Multiple sensitivity analyses yielded consistent estimates. Glofitamab significantly improved the RMST of 5 months (P = 0.007) over a 2-year follow-up.
CONCLUSIONS: In this robust comparative effectiveness study, short-ramp-up glofitamab administered at first relapse or progression after CAR T-cell failure significantly increases the chance of survival compared to contemporaneous non-bispecific salvage therapies, supporting its use as a preferred option for eligible patients with DLBCL who fail anti-CD19 CAR T-cell therapy.
TRIAL REGISTRATION: ClinicalTrials.gov identifier NCT04703686.

Zanubrutinib combined with R-CHOP in previously untreated diffuse large B-cell lymphoma with specific gene alteration: a phase II study.

Journal: Blood cancer journal | **Date:** 2026-Jul-01 | **PMID:** 42386710 | **DOI:** 10.1038/s41408-026-01553-4

Publication types: Journal Article

Abstract:

Diffuse large B-cell lymphoma (DLBCL) remains clinically heterogeneous, and outcomes with R-CHOP are suboptimal in patients carrying high-risk genomic alterations such as MYD88, CD79B, NOTCH1, TP53 mutations or MYC rearrangements. Many of these lesions are associated with aberrant BCR signaling, and prior studies in relapsed disease have suggested a degree of sensitivity to BTK inhibitors. These observations provided the rationale for evaluating zanubrutinib in the frontline setting for genomically defined high-risk disease. We conducted a prospective, single-center phase II study in previously untreated DLBCL patients aged 18-75 years with one or more of the above molecular features. To accommodate molecular testing time, patients received one initial cycle of R-CHOP, followed by five cycles of zanubrutinib plus R-CHOP (ZR-CHOP). Response was assessed using Lugano 2014 criteria. Primary endpoint is 3-year PFS; key secondary endpoints include ORR, OS, EFS, and safety. From February 2022 to April 2024, 59 patients were enrolled; 58 were evaluable for efficacy. Molecular subtyping identified 25 MCD-like, 16 A53-like, 3 N1-like, 10 other subtypes, and 4 MYC-rearranged cases. The ORR was 91.4% (53/58), with a CMR rate of 79.3% (46/58). MCD-like patients showed particularly favorable responses, with 84% achieving CMR. After a median follow-up of 30.1 months, the estimated 3-year PFS and OS were 80.3% (95%CI 70.5-91.5%) and 89.1% (95%CI 81.2-97.8%), respectively. MCD-like subgroup showed a 3-year OS of 100%, whereas N1-like had markedly poorer survival (2-year OS 33.3%). AEs occurred in 56/59 patients, mostly grade 1-2. Grade ≥ 3 events occurred in 57.6%, most commonly myelosuppression. No atrial fibrillation or other cardiac toxicities were observed. ZR-CHOP demonstrates encouraging activity and acceptable safety in molecularly selected high-risk DLBCL, particularly in the MCD-like subgroup. (ClinicalTrials.gov number, NCT05290337).

"Surveillance aid only. Verify every item against the primary source and apply clinical judgment and institutional protocols before any clinical use. Automated extraction can misread numbers, endpoints, and populations."